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Medical Policy

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Regence Medical Policy Update, March 1, 2017				
Changes to Regence Medical Policies Announced				
The Regence Group and its affiliated Plans use medical policies as guidelines for coverage decisions within the member's written benefits. Below are summaries of recent changes to The Regence Group's medical policies. The detailed policies and complete Medical Policy Manual are available online at www.regence.com. We have included the section and policy number for your convenience.				
Policy Name	Summary of Policy or Change	Section and Policy #	Coding / Implementation Change	PreAuthorization Change
Genetic Testing for Rett Syndrome	Specify the genes that may be medically necessary for the testing of Rett syndrome and include guidance on panel testing. Effective Date: June 1, 2017	Genetic Testing, Policy No. 68	N/A	N/A
Intensity Modulated Radiotherapy (IMRT) of the Prostate	Criteria added to define adverse pathologic findings and remove persistently detectable PSA levels. Added adaptive radiotherapy as investigational. Effective Date: June 1, 2017	Medicine, Policy No. 137	N/A	N/A
Endometrial Ablation	Added requirement for contraindication to non-contraceptive progestins. Effective Date: June 1, 2017	Surgery, Policy No. 01	N/A	N/A
Genetic Testing for Hereditary Hearing Loss	Created criteria for nonsyndromic (NSHL) and syndromic hearing loss and clarified the use of single gene testing and multigene panel testing. Effective Date: May 1, 2017	Genetic Testing, Policy No. 36	N/A	N/A
Transesophageal Endoscopic Therapies for Gastroesophageal Reflux Disease (GERD)	<i>This is a correction to what was reported for a 4/1/2017 effective date. Corrections will be effective as identified in the following columns.</i>	Surgery, Policy No. 110	Previously reported: "Added code 43236 with investigational denial." Corrected to now report: "Liberalized investigational denial on codes 43192 and 43201 to require Preauth effective 2/1/2017. Added code 43236 with preauth required effective 5/1/2017."	Preauth website updated with codes 43192 and 43201 effective 2/1/2017. Code 43236 will be added to the Preauth website effective 5/1/2017.
Cosmetic and Reconstructive Surgery	Added treatment session criteria to blepharoplasty. Modified panniculectomy criteria. Effective Date: April 1, 2017	Surgery, Policy No. 12	N/A	N/A
Spinal Cord and Dorsal Root Ganglion Stimulation	Added criteria for high frequency stimulation. Added language to the body of the policy to describe high frequency stimulation devices, including the Senza® system and corresponding HF10™ therapy. Effective Date: April 1, 2017	Surgery, Policy No. 45	Change HCPCS code C1822 from reviewed to investigational denial.	Change HCPCS code C1822 from reviewed to investigational denial.
Varicose Vein Treatment	Additional vein diameter measurement locations were added for ligation/stripping and endovenous ablation. For endovenous ablation, criterion added for clinical documentation that all incompetent segments of the same vein will be treated in the same session. Also, sclerotherapy for the upper extremities is considered investigational. Effective Date: April 1, 2017	Surgery, Policy No. 104	N/A	N/A
				N/A

Transesophageal Endoscopic Therapies for Gastroesophageal Reflux Disease (GERD)	Correction entered, see above entry for SUR110. Added code with investigational denial. Effective Date: April 1, 2017	Surgery, Policy No. 110	Added code 43236 with investigational denial.	
Femoroacetabular Impingement Surgery	Removed imaging requirement for radiologist review. Added physical therapy and intra-articular injections, unless contraindicated, to the conservative treatment requirements. Added additional language that requested procedures must be consistent with the documented anatomical abnormalities. Added capsular plication, capsular repair, labral reconstruction, iliobtibial band windowing, trochanteric bursectomy, abductor muscle repair, and/or iliopsoas tenotomy, when performed at the time of any FAI surgery, as components of and incidental to the FAI procedure. Effective Date: April 1, 2017	Surgery, Policy No. 160	N/A	N/A
Charged Particle (Proton or Helium Ion) Radiotherapy	Change to consider pediatric malignant solid tumors medically necessary and clarified the criteria for ocular, cervical spinal cord, and skull base tumors. Effective Date: March 1, 2017	Medicine, Policy No. 49	N/A	N/A
Intensity Modulated Radiotherapy (IMRT) of the Prostate	Change criteria to consider locally advanced prostate cancer, prior radiation to the planned target volume, and tumors close to organs at risk as medically necessary when policy criteria are met. Effective Date: March 1, 2017	Medicine, Policy No. 137	N/A	N/A
Endometrial Ablation	Clarified several criteria required and concomitant procedures. Updated contraindications to contraceptives to align with Centers for Disease Control and Prevention. Effective Date: March 1, 2017	Surgery, Policy No. 01	N/A	N/A
Gastroesophageal Reflux Surgery	Clarified hiatal hernia types in criteria for medically necessary and investigational indications. Effective Date: March 1, 2017	Surgery, Policy No. 186	N/A	N/A
Evaluating the Utility of Genetic Panels	Added additional investigational panels and updated the name of one panel. Effective Date: February 1, 2017	Genetic Testing, Policy No. 64	N/A	N/A
Analysis of Proteomic and Metabolomic Patterns for Early Detection of Cancer	Expanded the scope of the policy to address metabolomic tests which are considered investigational. Effective Date: February 1, 2017	Laboratory, Policy No. 41	N/A	N/A
Intensity Modulated Radiotherapy (IMRT) of the Head and Neck Cancers and Thyroid Cancer	Add criteria that IMRT is medically necessary when there is documented prior radiation treatment to the planned target volume. Clarified the language regarding the need for submitted histogram analysis. Effective Date: February 1, 2017	Medicine, Policy No. 138	N/A	N/A
Intensity-Modulated Radiotherapy (IMRT) of the Abdomen and Pelvis	Clarified the criteria language regarding the need for submitted histogram analysis. Effective Date: February 1, 2017	Medicine, Policy No. 139	N/A	N/A
Intensity Modulated Radiotherapy (IMRT): Central Nervous System (CNS) and Vertebral Tumors	Add criteria that IMRT is medically necessary when there is documented prior radiation treatment to the planned target volume. Clarified the language regarding the need for submitted histogram analysis. Effective Date: February 1, 2017	Medicine, Policy No. 147	N/A	N/A
Bariatric Surgery	Clarified reoperation criteria; added parietal cell separating gastrojejunostomy and AspireAssist device to investigational procedures; specified that sleeve gastrectomy and adjustable gastric banding are only medically necessary in the absence of GERD and takedown of fundoplication; separated out general criteria for reference throughout the policy. Provided clarification for hiatal hernia repair, specifying the procedure includes either repair of sliding or paraesophageal hernia. Effective Date: February 1, 2017	Surgery, Policy No. 58	Add preauth to code 43860. HCPCS code S2083 does not require preauth, but coverage is available only for members with bariatric surgery benefits.	Add code 43860 to the preauth list for this policy. Remove preauth on code S2083
			N/A	N/A

Assays of Genetic Expression in Tumor Tissue as a Technique to Determine Prognosis in Patients With Breast Cancer	Added EndoPredict and Breast Cancer Index tests to medical necessity criteria. Added additional language to clarify testing for patients with DCIS. Effective Date: January 6, 2017	Genetic Testing, Policy No. 42		
Administrative Guidelines to Determine Dental vs Medical Services	Criteria updated to reflect new contract language allowing general anesthesia coverage by a medical provider under the medical benefit when criteria are met. Effective Date: January 1, 2017	Allied Health, Policy No. 35	N/A	N/A
Genetic Testing for Inherited Susceptibility to Colon Cancer	Added criterion that Lynch syndrome testing may be medically necessary in endometrial cancer patients below age 50 when policy criteria are met. Effective Date: January 1, 2017	Genetic Testing, Policy No. 06	N/A	N/A
Genetic Testing for Familial Hypercholesterolemia	New policy for familial hypercholesterolemia genetic testing related to the PCSK9 inhibitor medications, which may require a definitive diagnosis of familial hypercholesterolemia. Effective Date: January 1, 2017	Genetic Testing, Policy No. 11	N/A	Add new policy to PreAuth website.
Analysis of Human DNA in Stool Samples as a Technique for Colorectal Cancer Screening	Fecal DNA testing using Cologuard® is now considered medically necessary once every three years for patients aged 50 to 85 years who have no signs or symptoms of colorectal disease and are at average risk for colorectal cancer. Fecal DNA testing with tests other than Cologuard® remains investigational. Effective Date: January 1, 2017	Genetic Testing, Policy No. 12	Termed investigational denial on CPT code 81528 and add preauth requirement.	Add CPT code 81528 to PreAuth website.
Urine Drug Testing for Substance Abuse and Chronic Pain	Three presumptive testing codes have been deleted with the annual code update process, and three comparable codes have been added to replace. Effective Date: January 1, 2017	Laboratory, Policy No. 68	Added new codes 80305, 80306, 80307, and G0659 with 15 unit per year limit.	N/A
Intensity Modulated Radiotherapy (IMRT) of the Thorax	Significant reformatting and clarifications made to the policy criteria. New requirements for histogram analysis and the term curative treatment was added to the criteria for specific indications. IMRT may be considered medically necessary for individuals with large breasts in order to avoid or minimize hot spots when policy criteria is met. A list of information needed for review was updated in the Policy Guidelines section. Effective Date: January 1, 2017	Medicine, Policy No. 136	N/A	N/A
New and Emerging Medical Technologies and Procedures	Update the policy in accordance with the annual code update. Moved code 0404T to Medical Policy, Radiofrequency Ablation for Tumors, No. SUR92. Effective Date: January 1, 2017	Medicine, Policy No. 149	N/A	N/A
Coverage of Treatments Provided in a Clinical Trial	Clarified the criteria and added a list of information needed for review. Effective Date: January 1, 2017	Medicine, Policy No. 150	N/A	N/A
Transgender Services	Revisions have been made to the not medically necessary services. Drugs for hair loss or growth have been removed from the policy. Breast augmentation, hair removal, hair transplantation, mastopexy, and nipple/areola reconstruction in the absence of concurrent or prior subcutaneous or simple/total mastectomy may be considered medically necessary when criteria are met. Effective Date: January 1, 2017	Medicine, Policy No. 153	Changed Criteria B and C in that if the listed services are billed for gender identity disorders they will be denied as not medically necessary (provider write-off) and not billed for gender identity disorder the services will continue to be denied as cosmetic denials (member balance).	Added 15775, 15776, and 17380 to PreAuth website.
Spinal Cord and Dorsal Root Ganglion Stimulation	Added criteria for dorsal root ganglion stimulation. Effective Date: January 1, 2017	Surgery, Policy No. 45	Add HCPCS code C1820 for review.	Name changed on PreAuth website.
Ventricular Assist Devices and Total Artificial Hearts	Clarified that aortic counterpulsation devices are included in investigational criteria, as they are not FDA-approved. Effective Date: January 1, 2017	Surgery, Policy No. 52	N/A	N/A
Deep Brain Stimulation	Policy criteria reorganized with no significant changes.	Surgery, Policy No. 84	N/A	N/A

	Effective Date: January 1, 2017			
Radiofrequency Ablation of Tumors (RFA)	Clarified that RFA for treatment of uterine fibroids is investigational.	Surgery, Policy No. 92	N/A	N/A
	Effective Date: January 1, 2017			
Varicose Vein Treatment	Policy reorganized and grouped together into general criteria, procedures, and treatment sessions. A list of information needed for review was updated in the Policy Guidelines section. Additional clarifications were added throughout the criteria.	Surgery, Policy No. 104	N/A	N/A
	Effective Date: January 1, 2017			
Implantable Bone-Conduction and Bone-Anchored Hearing Aids	Consider partially-implantable bone-conduction (bone-anchored) hearing aid (s) to be medically necessary when criteria are met.	Surgery, Policy No. 121	N/A	N/A
	Effective Date: January 1, 2017			
Cryosurgical Ablation of Miscellaneous Solid Organ, Pulmonary, and Breast Tumors	Clarified the policy scope to explicitly state pulmonary tumors are included.	Surgery, Policy No. 132	N/A	Name changed on PreAuth website.
	Effective Date: January 1, 2017			
Applied Behavior Analysis for the Treatment of Autism Spectrum Disorders	Expand intervals for evaluation from every 'three to six months' to every 'three to twelve months'.	Behavioral Health, Policy No. 18	Removed codes H0031, H0032, H0046, H2012, H2014, H2019, and S5111 from the medical policy and term current edits. A workflow will be created to deny codes H0031, H0032, H0046, H2014, and S5111 for Autism diagnoses and allow other diagnoses to pay. The edit changes will be effective 1/1/2017.	N/A
	Effective Date: December 1, 2016			
Genetic Panel Testing (5-50 genes) for Hematolymphoid Neoplasms or Disorders	New policy implementation for targeted multiplex genetic sequence analysis panels using DNA/RNA analysis for hematolymphoid neoplasms or disorders comprised of at least five and up to 50 genes. These panels may be used to confirm a diagnosis. This policy will list medically necessary panels related to hematolymphoid neoplasms or disorders, such as myeloid neoplasms and acute leukemias. At this time, the policy contains one panel.	Genetic Testing, Policy No. 09	Adding unlisted codes 81479 and 81599 to this new policy and keeping existing edits. Add code 81450 to the new policy and term the current investigational denial and add preauth.	Add policy to the preauth website for code 81450.
	Effective Date: December 1, 2016			
Evaluating the Utility of Genetic Panels	Added additional investigational panels and removed several panels.	Genetic Testing, Policy No. 64	N/A	N/A
	Effective Date: December 1, 2016			
Intensity Modulated Radiotherapy (IMRT) of the Head and Neck Cancers and Thyroid Cancer	IMRT for anaplastic thyroid carcinoma and locoregional recurrence may be considered medically necessary. In addition, IMRT for treatment of other thyroid cancers may be considered medically necessary when the tumor is in close proximity to organs at risk and dose/volume histograms demonstrate the need for IMRT. The policy title was updated to reflect the newer terminology of radiotherapy.	Medicine, Policy No. 138	N/A	N/A
	Effective Date: December 1, 2016			
Radioembolization for Primary and Metastatic Tumors of the Liver	Specified that hepatocellular carcinoma (criteria I.B.) and intrahepatic cholangiocarcinoma (criteria I.E.) must be primary.	Medicine, Policy No. 140	N/A	N/A
	Effective Date: December 1, 2016			
Intensity Modulated Radiotherapy (IMRT): Central Nervous System (CNS) and Vertebral Tumors	Change term in title from "Radiation Therapy" to "Radiotherapy". Change criteria to only allow IMRT when the tumor is in close proximity to organs at risk. Added a Policy Guidelines section with a list of information needed for review, definition of at-risk organs, and an IMRT dose constraint reference.	Medicine, Policy No. 147	N/A	N/A
	Effective Date: December 1, 2016			
Percutaneous Angioplasty and Stenting of Veins	Expanded medically necessary criteria to include superior vena cava syndrome as a result of non-malignant causes or from intrinsic stenosis/occlusion.	Surgery, Policy No. 109	N/A	N/A

	Effective Date: December 1, 2016			
Adoptive Immunotherapy	New policy to address the use of adoptive immunotherapy as a treatment for a number of indications. Adoptive immunotherapy, regardless of cell type, is considered investigational for all indications. Effective Date: November 1, 2016	Medicine, Policy No. 42	Add investigational denial to code S2107 and review on code 36511 to help identify service billed when provider is not using S2107.	N/A
Intensity Modulated Radiotherapy (IMRT) of the Prostate	One addition to the policy requiring 90-day notification that states IMRT for treatment of prostate cancer, including palliative treatment, at radiation doses less than 60 Gy is considered not medically necessary. Effective Date: November 1, 2016	Medicine, Policy No. 137	N/A	Code is already on the PreAuth list
Cochlear Implant	Criteria changed to consider hybrid cochlear implant/hearing aid devices medically necessary when criteria are met. In addition, cochlear implants may be considered medically necessary in individuals with a diagnosis of enlarged vestibular aqueduct when criteria are met. Effective Date: November 1, 2016	Surgery, Policy No. 08	N/A	N/A
Electrical Bone Growth Stimulators (Osteogenic Stimulation)	Reorganized the criteria to combine noninvasive indications. Effective Date: October 1, 2016	Durable Medical Equipment, Policy No. 83.11	N/A	N/A
Intensity-Modulated Radiotherapy (IMRT) of the Abdomen and Pelvis	Clarified the criteria for abdomen and pelvis which states that IMRT may be considered medically necessary when the tumor is in close proximity to organs at risk and dose/volume histograms demonstrate a need for IMRT. The title was updated to reflect the newer terminology of radiotherapy. A list of information needed for review was updated to align with the revised policy criteria. Effective Date: October 1, 2016	Medicine, Policy No. 139	N/A	N/A
New and Emerging Medical Technologies and Procedures	Code 0402T [Collagen Crosslinking with Riboflavin (CXL, C3-R, CCR, CCL)] has been removed from the policy and will be addressed in a new medical policy, Medicine 159. Effective Date: October 1, 2016	Medicine, Policy No. 149	Move code 0402T to medical policy MED159.	N/A
Corneal Collagen Cross-Linking	New policy to address corneal collagen cross-linking (CXL) procedures as a treatment for various corneal conditions. Effective Date: October 1, 2016	Medicine, Policy No. 159	Add preauth to code 0402T.	Add policy to the preauth website for code 0402T.
MRI-Guided Focused Ultrasound (MRgFUS)	Clarified our investigational criterion on the use of MRgFUS for the treatment of movement disorders. Effective Date: October 1, 2016	Surgery, Policy No. 139	N/A	N/A
Implantable Sinus Stents for Postoperative Use Following Endoscopic Sinus Surgery and for Recurrent Sinonasal Polyposis	Criteria clarified that stents to treat recurrent polyposis are investigational. Effective Date: October 1, 2016	Surgery, Policy No. 198	N/A	N/A
The following is a list of recently <u>archived</u> policies:				
Computed Tomography (CT) Perfusion Imaging of the Brain		Archive Effective Date: March 1, 2017		Radiology, Policy No. 54
Thermal Capsulorrhaphy as a Treatment of Joint Instability		Archive Effective Date: March 1, 2017		Surgery, Policy No. 100
Saturation Biopsy for Diagnosis and Staging of Prostate Cancer		Archive Effective Date: February 1, 2017		Surgery, Policy No. 170
Transvaginal and Transurethral Radiofrequency Tissue Remodeling for Urinary Stress Incontinence		Archive Effective Date: January 1, 2017		Surgery, Policy No. 130
Enhanced External Counterpulsation (EECP)		Archive Effective Date: October 1, 2016		Medicine, Policy No. 66
Acoustic Cardiography		Archive Effective Date: October 1, 2016		Medicine, Policy No. 114

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The Blue Cross and/or Blue Shield Plans comprising The Regence Group serve Idaho, Oregon, Utah and much of Washington state. The Regence Group and each of its affiliate Plans are independent licensees of the Blue Cross and Blue Shield Association.